

When a woman with type 1 or type 2 diabetes mellitus gets pregnant

Diabetes in pregnancy

Diabetes in pregnancy occur in the following situations;

- When a woman with type 1 or type 2 diabetes mellitus gets pregnant
- When a pregnant woman develops gestational diabetes

How do you continue treating Type 1 or Type 2 Diabetes during pregnancy

Treatment should be initiated by a medical doctor and a clinical care team.

For pregnant women with type 1 diabetes, the insulin being used before pregnancy will be continued. But if it is not reducing the blood sugar to normal levels, the doctor might consider changing to improve blood sugar control.

For pregnant women with type 2 diabetes, the blood sugar lowering medications used before pregnancy will be checked. Medications that are not safe for the baby will be replaced with either insulin and/or metformin.

Depending on the blood sugar level at the time of starting ante-natal care, the doctor may recommend the following steps;

Diet and Physical Activity to control blood sugar before pregnancy: First, physical activity and diet will be recommended for pregnant women with diabetes who have achieved blood sugar control with physical activity and diet before pregnancy. Then the fasting blood sugar is monitored to see if this works. If it works, it is continued during pregnancy. But if it does not work, then medications that lower blood sugar will be recommended.

Treatment with medications before pregnancy: Medications that are safe for the baby will be recommended to lower blood sugar. Only two drugs are considered safe for both the baby and the pregnant woman. It is either metformin or insulin that will be recommended. Depending on the situation, one drug may be recommended first. This may be either insulin or metformin, if one of those drugs work, it is continued.

But If one drug which is either metformin or insulin does not work, then another medication is added. If insulin was used at first, then metformin is added. If metformin was used at first, then insulin is added.

What kind of food is recommended for diabetes in pregnancy

A balanced, healthy diet is recommended to manage diabetes.

Eat about 5-6 servings of non-starchy vegetables every day.

Eat more of whole grains, unsaturated fat, legumes, and lentils.

Avoid sugar, limit saturated fat or trans fats and reduce salt intake. Avoid processed carbs like bread and other pastries.

Avoid alcohol and smoking during pregnancy.

What kind of exercise is recommended for diabetes in pregnancy

Exercise is important for reducing blood sugar. It is important to combine aerobic exercise and strength training exercises during pregnancy. The intensity of exercise depends on your health status. It is important to discuss with your doctor before you start exercising.

About 150 minutes of moderate-intensity exercise is recommended a week. It is also important to include muscle-strengthening and stretching exercises to improve or maintain strength and flexibility during pregnancy.

How does a pregnant woman with diabetes manage weight during pregnancy

Gaining some weight during pregnancy is very important. The amount of weight a pregnant woman gains depends on her weight before pregnancy. The table below shows the normal weight gain to expect. If you gain too much weight during pregnancy, discuss it with your doctor.

	Normal weight gain per week		Total weight gain	
	Kg/week	lb/week	Kg	lbs
BMI less than 18.5	0.5	1.0	12.5 - 18	28 - 40
BMI 18.5 to 24.9	0.4	1.0	11.6 - 16	25 - 35
BMI 25 to 29.9	0.3	0.6	7 – 11.5	15 - 25
BMI more than 30	0.2	0.5	5 - 9	11 -20

How does a pregnant woman with diabetes monitor her blood sugar

Monitoring blood sugar regularly is very important. This helps you to know when it is too high and when it is too low. Checking at home with a glucometer is recommended. If you don't have a glucometer, you should monitor whenever you go for ante-natal care.

Monitoring before and after meals can help you recognize which foods increase your blood sugar and avoid them.

If you are using insulin, you should also consider monitoring more often.

HBA1C will be recommended in the first trimester as soon as you start ante-natal care to know how well your blood sugar control is doing. This guides the doctor to make recommendations on your target for blood sugar control and the type of drugs to give you.

If you are living with Type 1 diabetes, a continuous glucose monitoring (CGM) system is recommended if you can afford it. If you are unable to afford it, please use normal glucometer to check your blood sugar as often as you can.

How should a pregnant woman with diabetes monitor the baby

It is important to monitor how well the baby is developing. This can be done with ultrasound scan. It is recommended to have ultrasound scans before 24 weeks to find out how far along the pregnancy is. The ultrasound scan will help to detect if the baby has any abnormalities.

You also need to monitor the movements the baby is making to know when it normal movements or when it reduces or increases. It is important to notify your doctor once you notice abnormal movements.

You may need to use ultrasound scan to monitor more often as you approach your expected delivery date.

Ante-natal care advice for a woman with diabetes before pregnancy

If you are a pregnant women with type 1, type 2 before pregnancy will need to start ante-natal care as soon as you discover that you are pregnant. You need to tell the doctors and nurses about your health. This will help them develop a care plan that will address your needs.

In addition to general topics on how to stay healthy during pregnancy, you will also need to cover;

- the effects of diabetes in pregnancy on maternal, fetal, newborn and child health outcomes;
- diet and physical activity;
- appropriate weight gain during pregnancy;
- how to manage your blood sugar;

You will also need frequent monitoring to check if the baby is growing well and how well you are keeping your blood sugar under control within normal limits.

Additional screening for complications of diabetes during pregnancy

It is important to screen for complications of diabetes during pregnancy. This will include checking for damage to the eyes (retinopathy) and damage to the kidneys. These are common complications that may be worsened by pregnancy.

If they are detected, specialist care will be needed to reduce these complications during pregnancy.

How should a pregnant woman with diabetes prepare for delivery.

Follow the guidance of the doctor, midwife and clinical care team to prepare for delivery. At the beginning of ante-natal care, they will calculate your expected delivery date (EDD). Take note of your EDD. As the EDD approaches, the doctor and the care team, will provide additional guidance.